Document Control

Title

Protocol for Non Traumatic Chest Pain of Suspected Cardiac Origin in Minor Injury Units for Patients over 18 years old

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Directorate Emergency Services, Logistics and Resilience	**Department** Emergency Department

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0.1	May 2019	Draft	Initial version for consultation
1.0	July 2019	Final	Approved by (ED Lead Clinician) and (Medical Director) May 2019 and published on Bob. Ratified at Drugs & Therapeutics Committee.

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Lead Director Medical Director	
Document Class Protocol	**Target Audience** Nursing, MIU Staff
Distribution List Senior Management	**Distribution Method** Trust's internal website
Superseded Documents	

Issue Date	Review Date	Review Cycle
July 2019	July 2022	Three years

Approval and Review Process

- Lead Clinician for Emergency Department

Consulted with the following stakeholders: (list all)

- MIU Practitioners
- ED Consultants NDHCT

Local Archive Reference
G:\Policies and Protocols
Local Path
MIU
Filename
Protocol for Chest Pain (non-traumatic)

Policy categories for Trust's internal website (Bob) MIU Standard Operating Procedures	**Tags for Trust's internal website (Bob)** None

1. Table of Contents

Document Control 1
2. Purpose 3
3. Presenting Symptoms 3
4. History 4
5. Discharge Pathway 5
6. References 6
7. Associated Documentation 6
APPENDIX A – Essential Documentation for All Patients Attending Unit or Centre 7
APPENDIX B – Essential Documentation for All Patients Attending Unit or Centre 8
APPENDIX C – Training Competency Form 9

2. Purpose

This Protocol is for the use by staff employed by Northern Devon Healthcare Trust who have achieved the agreed clinical competencies to work under this protocol.

3. Presenting Symptoms

- Chest pain
- Shortness of breath
- Dizziness
- Lethargy
- Cough
- Pale
- Clammy / Diaphoresis
- Anxiety
- Palpitations
- Nausea / vomiting
- Haemoptysis
- Confusion
- Pain radiation to neck/mandible /shoulder/ back

Assess immediately using the ABCDE approach and record a 12 lead ECG

- Respiratory rate and SpO2
- Bilateral BP's – systolic variation may indicate aortic aneurysm
- Heart rate and rhythm and capillary refill time
- Glasgow coma score and blood glucose measurement (BM) and temperature

Arrange emergency transfer to secondary care:

All patients with clinical features which suggest a serious cause of chest pain such as:

- Respiratory rate > 30 beats per minute.
- Tachycardia of > 130 beats per minute.
- Systolic blood pressure < 90mmHg or diastolic blood pressure < 60mmHg.
- Oxygen Saturations < 92% or central cyanosis.
- Altered level of consciousness.
- High temperature, especially > 38.5 degrees.
- Suspected acute coronary syndrome (ACS) who have current chest pain, any signs of complications (such as pulmonary oedema).
- Persons who are pain free but have had chest pain in the last 12 hours and have an abnormal ECG.

- Persons who have a recent history of ACS who develop further chest pain.

Consider referring persons with suspected ACS if pain has resolved.v

4. History

Take a history while waiting for transfer and document.

4.1. Pain

- Pain score
- Site
- Radiation
- Frequency
- Severity
- Onset
- Duration
- Time of worse pain
- Any medications taken
- Pain on exertion
- Pain on inspiration
- Known cardiovascular disease

4.2. ACS Risk Factors

- Age
- Smoking
- Dyslipidaemia
- Diabetes mellitus
- Obesity
- Family history
- Use of cocaine / recreational drugs

Other risk factors:

- Anaemia
- Aneurysm
- Aortic dissection
- Marfan's disease
- Kawasaki disease

4.3. Initial Emergency Treatment prior to transfer:

- Sit the person up
- Do not routinely administer oxygen. Only offer supplementary oxygen to persons with an oxygen saturation (SpO2) < 94% who are not at risk of hypercapnic respiratory failure.
- Use an O2 face mask. Adjust the flow rate to 5-10 litres per minute to achieve a target SpO2 of 94-98.

- For persons with chronic obstructive airways disease who are at risk of hypercapnic respiratory failure use a 28% venturi oxygen mask and keep the flow at 4litres per minute to achieve a target SpO2 of 88-92%.
- If the person has suspected ACS treat the pain with glyceryl trinitrate (GTN) as per PGD.
- Administer aspirin 300mg as per PGD unless there is clear evidence that the person is allergic to it.
- Send a written record of the administration with the patient to secondary care.
- Send the results of the ECG to the receiving hospital.
- Record NEWS2 score regularly until transfer and send documentation with the transfer.
- Insert a cannula if competent to do so and take and send bloods for U/E, FBC, troponin.

DO NOT DELAY HOPITAL TRANFER

5. Discharge Pathway

5.1. DO NOT DISCHARGE PATIENTS WITH CURRENT CHEST PAIN

Assess and document pain score prior to discharge and arrange follow up with GP.

DOCUMENTATION TO BE COMPLETED

- Clinical treatment record as per Documentation & record keeping policies. Copy of clinical treatment record to General Practitioner; to be sent to surgery as per Record keeping policy.
- For patients being transferred to secondary care, ensure a copy of the clinical treatment record is sent with patient. A copy will also be sent to surgery in normal manner.

5.2. BEFORE DISCHARGE ENSURE

- Those patients who have been referred for further acute intervention have appropriate transport to meet their needs, all relevant treatment has been prescribed and administered and correct information and documentation is given to the patient.
- The patient understands that if condition deteriorates or they have further concerns they should seek further advice.

- The patient demonstrates understanding of advice given during consultation.
- The patient demonstrates an understanding of how to manage subsequent problems.

6. References

National Institute for Clinical Excellence: Chest pain (January 2017)

Emergency Department guideline NDHCT (2012)

7. Associated Documentation

7.1. British national Formulary online edition via

7.2. https://www.new.medicinescomplete.com/

7.3. Consent Policy (2018) NDHCT

7.4. Medicines Policy (2018) NDHCT

7.5. Patient Group Direction Policy (2016) NDHCT

7.6. Patients at Risk of Deterioration Policy V4.0 (2019)

APPENDIX A – Essential Documentation for All Patients Attending Unit or Centre

Adults Consent
Gain consent to be seen by a nurse practitioner
Gain consent for treatment and sharing information and document.

Clinical Presentation
If unwell assess for:

- Airway
- Breathing
- Circulation
- Disability
- Exposure

Document a full set of observations including neurological observations including Glasgow Coma Score if applicable.

Record NEWS 2 for handover to secondary care.

Document pain score using numeric rating scale. For cognitively impaired patients document any signs of pain (e.g. grimaces or distress).

Safeguarding:

- Ask the domestic abuse question, 'do you feel safe at home?'
- Assess for learning disability and whether patient has a hospital passport in place.
- Assess for mental capacity and if the person is a vulnerable adult.
- Assess for falls risk. Complete falls referral if applicable.

APPENDIX B – Essential Documentation for All Patients Attending Unit or Centre

Child and Young Persons under 18 Years old are not treated under this protocol.

APPENDIX C – Training Competency Form

Protocol for Chest Pain (non-traumatic)

Protocol operational from July 2019 and expires end of July 2022

The registered health professional named below, being employees of Northern Devon Healthcare Trust based at ……. have received training and are competent to operate under this protocol

NAME *(please print)*	PROFESSIONAL TITLE	SIGNATURE	AUTHORISING MANAGER *(please print)*	MANAGER'S SIGNATURE	DATE

Keep original with the authorising manager and send a copy to: Karen Watts, Emergency Department, Northern Devon Healthcare Trust NHS, Raleigh Park, Barnstaple, Devon, EX31 4JB